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BRIEF REPORT

Relations Between Social Camouflaging, Life Satisfaction, and Depression Among Polish Women with ADHD

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Abstract

Background: This study investigated the relationship between social camouflaging, life satisfaction, and depression symptoms in Polish women with ADHD. It aimed to fill a research gap in understanding ADHD manifestations in women, particularly focusing on social camouflaging—a concept known in autism, referring to strategies used to mask symptoms in social interactions. *Methods*: A total of 329 women with ADHD took part in an online survey. The survey measured ADHD symptoms using the ADHD Self Report Scale (ASRS-vI.I), life satisfaction with the Satisfaction With Life Scale (SWLS), and depressive symptoms using the Patient Health Questionnaire-9 (PHQ-9). Additionally, a novel set of questions, inspired by the Camouflaging Autistic Traits Questionnaire (CAT-Q) but tailored for ADHD, was utilized to assess social camouflaging. *Results*: Results indicated a significant negative association between social camouflaging and life satisfaction and a positive correlation with depressive symptoms, even after controlling for demographic variables. Additionally, factors like being in a relationship and having a positive subjective financial situation correlated with higher well-being. *Limitations*: Reliance on self-report measures, the cross-sectional design, and participants' self-reported ADHD diagnosis are the main limitations of the study. *Conclusion*: The research underscores the importance of social camouflaging in understanding ADHD in women, suggesting that efforts to mask symptoms and meet societal expectations may link to lower life satisfaction and increased depressive symptoms. These findings advocate for continued research into these dynamics to develop more effective support for women with ADHD.

Keywords ADHD · Life satisfaction · Depression · Social camouflaging

Introduction

The underrepresentation of females in Attention Deficit Hyperactivity Disorder (ADHD) research has resulted in a significant gap in understanding this condition's unique aspects in women, including their specific manifestations, developmental trajectories, and the most effective assessment and treatment strategies (Attoe & Climie, 2023; Chronis-Tuscano, 2022). This oversight has left many girls and women with ADHD undiagnosed and untreated, as most research and clinical practices have focused on male, elementary school-aged populations. Recent shifts in ADHD research, highlighted in the Annual Research Review (Hinshaw et al., 2022), are now increasingly directed towards studying ADHD in women. There is also a growing recognition that girls and women may camouflage their ADHD symptoms, a phenomenon previously identified in autism research, particularly in autistic females, and now considered relevant for ADHD (Agnew-Blais & Michelini, 2023; Hinshaw et al., 2022; Schuck et al., 2019; Tubío-Fungueiriño et al., 2021).

Our study seeks to address the gap in understanding ADHD in women by focusing on social camouflaging that can be defined as the behaviors and strategies that individuals with ADHD use to hide, mask, or compensate for their ADHD symptoms in social situations (Hull et al., 2017, 2019; Mylett, 2022; van der Putten et al., 2024). This process is aimed at conforming to neurotypical expectations and may involve using strategies to minimize the visibility of symptoms of neurodevelopmental condition

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and conform to societal expectations (Hull et al., 2017, 2019; Mylett, 2022; van der Putten et al., 2024). Previous studies have identified complex relations between social camouflaging and mental health difficulties in autistic individuals suggesting that the act of camouflaging, while serving as a coping mechanism for navigating social interactions, can paradoxically exacerbate mental health difficulties (Field et al., 2024). Although camouflaging has been previously examined mostly in the context of autism, this phenomenon is highly pertinent to ADHD (Mylett, 2022; van der Putten et al., 2024), especially considering the associated psychosocial pressures and stigma that affect not only adults but individuals at all ages (Lebowitz, 2016). These pressures include reluctance to socially interact with individuals diagnosed with ADHD, often stemming from misconceptions that they are unreliable, less trustworthy, or careless (Fuermaier et al., 2012). Furthermore, stigma manifests in various forms, such as the belief that adult ADHD is not a legitimate disorder, with some suggesting that symptoms are simulated or that the condition is fabricated by pharmaceutical companies (Masuch et al., 2019). Consequently, individuals with ADHD may employ social camouflaging techniques to avoid encountering these stereotypes and mitigate negative social repercussions. Van der Putten and colleagues' comparative analysis (2024) has revealed that adults with ADHD, like those with autism, engage in camouflaging behaviors, however to a lesser extent. Their work underscores the necessity for nuanced measures to accurately capture camouflaging behaviors across neurodevelopmental conditions.

In the current study, we investigate the relationship between social camouflaging, depressive symptoms, and life satisfaction in Polish women with ADHD, in light of the high comorbidity with internalizing disorders such as depression and anxiety. Our research, conducted with Polish women, aligns with the increasing call for more inclusive and diverse ADHD studies, emphasizing the importance of research across various countries and cultures (Chronis-Tuscano, 2022; Hinshaw et al., 2022). The focus on this population is informed by the authors' backgrounds and a notable deficiency in research on ADHD within the Polish context, where neurodevelopmental disorders, including ADHD and ASD, are often misunderstood and underdiagnosed (Lenart & Pasternak, 2023; Szaniawska, 2010). Recent discussions in Poland highlight significant challenges in the diagnosis and treatment of ADHD (Pankowski & Wytrychiewicz-Pankowska, 2024). Medications are frequently inaccessible and reimbursable only for individuals under 18 years, despite the growing acknowledgment of adult ADHD in Poland. Furthermore, the visibility of the topic of ADHD through social media is increasing, significantly influencing public perception and awareness (Karteczka, 2012). This increased awareness sharply contrasts with ongoing healthcare challenges, underscoring the gap between public engagement and actual support for individuals with ADHD. Additionally, there is a discrepancy between government claims of ADHD overdiagnosis and what research indicates to be a persistent underdiagnosis and a lack of adequate treatment options (Buźniak et al., 2022; Pankowski & Wytrychiewicz-Pankowska, 2024). This context underscores the need to better understand the predictors of well-being for individuals with ADHD living in such complex sociocultural environments.

Building on previous research, our study extends the findings of van der Putten and colleagues (2024), who investigated camouflaging behaviours using the Camouflaging Autistic Traits Questionnaire (CAT-Q; Hull et al., 2019), initially designed for autistic individuals. We have developed the Social Camouflaging of ADHD Symptoms Scale, a 10-item questionnaire adapted from the Camouflaging Autistic Traits Questionnaire (CAT-Q; Hull et al., 2019) and tailored for ADHD. We hypothesize that social camouflaging will be positively associated with depressive symptoms and inversely related to life satisfaction in women with ADHD, controlling for the intensity of ADHD symptoms and various demographic and diagnosis-related variables. The demographic questions, as well as those regarding ADHD diagnosis and comorbid conditions, were chosen due to their potential impact on the well-being of women with ADHD. Our aim in controlling for these variables was to distinguish the unique association between social camouflaging and depressive symptoms and life satisfaction beyond other variables like age, education, and mental health status.

Methods

Participants and Procedure

The data were collected from 28 March 2023 to 6 September 2023 via an online survey posted on two Polish Facebook groups for people with ADHD. These groups consisted of individuals with ADHD and their family members and significant others. Given the substantial size of these groups, which collectively include up to 50,000 users, it is reasonable to assume a diverse range of ages, educational backgrounds, and socioeconomic statuses among the members. The recruitment messages were designed to be clear and accessible to ensure understanding across various educational levels. However, despite these efforts to include participants from

diverse backgrounds, the majority of the participants were women with a higher education degree and from big cities. When the survey link was posted it was specified that the only people with an ADHD diagnosis who are over the age of 18 are invited to participate in the study. Informed consent was provided by participants through the survey platform. Prior to initiating the survey, participants were presented with detailed information that described the purpose, content, and estimated duration of the survey. This introduction also emphasized that participation was entirely voluntary and that participants could withdraw from the study at any time without any repercussions. This information ensured that participants were fully aware of the study details and their rights before they proceeded. We assessed ADHD symptoms through the ADHD Self Report Scale (ASRS-vI.I) to make sure that the recipients actually met the screening criteria for an ADHD diagnosis. The final analysis included 329 Polish women (mean age of 30.82 and standard deviation of 8.02) who reported having a psychiatric diagnosis of ADHD and scores on the ASRS scale above a cut-off indicative of ADHD (Kessler et al., 2005). Demographics of the sample are presented in Table 1.

Table 1 Demographic characteristics of the sample

Demographic variables	*N*	Percent
Age (years)		
18–22	47	14.0%
23–27	80	24.0%
28–32	80	24.0%
33–37	60	18.0%
38–42	28	9.0%
43–47	24	7.0%
48–52	5	2.0%
53–58	5	2.0%
Place of residence		
Countryside	25	7.6%
City with up to 50 000 residents	31	9.4%
City with 50 000–150 000 residents	25	7.6%
City with 150 000–500 000 residents	27	8.2%
City with more than 500 000 residents	221	67.2%
Marital status		
Single	77	23.4%
Dating	151	45.9%
Married	86	26.1%
Divorced/Separated	14	4.3%
Widow	1	0.3%
Occupational status		
Student	73	22.2%
Unemployed	19	5.8%
Employment contract	163	45.7%
Contract for work	12	3.6%
Mandate contract	55	16.7%
Own business	58	17.6%
Other	19	5.8%
Nationality		
Polish	323	98.2%
Other	6	1.8%
Education		
Primary education	3	0.9%
Secondary education	61	18.5%
Vocational education	3	0.9%
Incomplete higher education	77	23.4%
Higher education	185	56.2%

Measures

ADHD Self Report Scale (ASRS-vI.I)

The ADHD Self Report Scale (ASRS-vI.I; Kessler et al., 2005) is an 18-item instrument designed to measure ADHD symptoms experienced over the previous six months. Participants provide ratings of symptoms' frequency that contribute to an overall score ($\alpha = 0.73$ in the current study), which can be further analyzed through two nine-item subscales for Inattention (INA; $\alpha = 0.63$) and Hyperactivity-Impulsivity (HI; $\alpha = 0.68$). A specific cutoff point, derived from the ASRS, identifies scores indicative of ADHD, facilitating diagnostic screening.

Satisfaction with Life Scale (SWLS)

The Satisfaction With Life Scale (SWLS; Pavot & Diener, 2008) employs a 5-item questionnaire to assess an individual's overall satisfaction with their life score ($\alpha = 0.87$ in the current study). Participants respond using a 7-point Likert scale, with aggregated item scores forming a composite measure of life satisfaction. This scale offers a global assessment of subjective well-being.

Patient Health Questionnaire-9 (PHQ-9)

The Patient Health Questionnaire-9 (PHQ-9; Kroenke et al., 2001) is a nine-item self-report inventory used to assess the presence and severity of depressive symptoms within the last two weeks ($\alpha = 83$ in the current study). Responses are scored on a scale ranging from 0 (not at all) to 3 (nearly every day), allowing for a quantitative evaluation of depression severity.

Social Camouflaging of ADHD Symptoms Scale

To assess social camouflaging in individuals with ADHD, we developed a novel 10-item questionnaire inspired by the Camouflaging Autistic Traits Questionnaire (CAT-Q; Hull et al., 2019) and existing literature on camouflaging in ADHD (Mylett, 2022). The questions were designed

to be relevant to camouflaging ADHD symptoms (e.g. *I put a lot of effort into controlling my behavior and excessive energy; I put a lot of effort into trying not to talk too much and interrupt others)* or general and relevant to camouflaging symptoms of various neurodevelopmental disorder (for this purpose a few items from CAT-Q were chosen e.g. *In social situations, I feel like I'm performing rather than being my true self*). The items were consulted with two clinicians who are also researchers specializing in clinical psychology, with experience in the clinical work with individuals with ADHD. Before conducting this research we ran a pilot survey to assess the understandability of the questions of the Social Camouflaging scale. This scale uses a 7-point Likert scale for responses, with higher scores indicating greater engagement in camouflaging behaviors. This instrument demonstrates a one-factor structure (Table S1 in supplementary material) and high reliability ($\alpha = 0.91$). Additionally, we have calculated the mean and standard deviation for each item (shown in Table S2 in supplementary material) to verify that the items effectively differentiate among participants, ensuring that the scale accurately measures varying levels of camouflaging behaviors.

Data Analysis

We conducted the analyses using SPSS version 29. The hierarchical regression was conducted with life satisfaction as a dependent variable, followed by depressive symptoms as a dependent variable. In the first step of our hierarchical regression, we incorporated key demographic factors and clinical diagnoses to establish a baseline model of factors that may affect the well-being of individuals with ADHD. This included participant's age, age of diagnosis, diagnosed ASD, diagnosis of depression, anxiety disorders or personality disorders, lack of psycho- and pharmacotherapy, subjective evaluation of the financial situation, and being in a relationship. In the next step, we added ADHD-specific symptoms of hyperactivity-impulsivity and inattention to isolate their unique effects beyond those of the demographic and diagnosis-related factors. Finally, we introduced social camouflaging symptoms to explore their specific association with the outcomes, adjusting for all previously included variables. Several key assumptions were initially tested. Normality of residuals was assessed using a normal P-P plot, which revealed that the data points closely followed the theoretical diagonal line, indicating no significant deviations from normality. Homoscedasticity was evaluated through a scatterplot of residuals, where the dispersion of points showed no apparent pattern, resembling a random scatter and thus satisfying the homoscedasticity criterion. Lastly, the absence of multicollinearity was confirmed by examining the Variable Inflation Factors (VIF) obtained through collinearity diagnostics. All VIF values were below the commonly accepted threshold of 10 (Darlington & Hayes, 2016), indicating that multicollinearity did not affect the analysis.

Results

Descriptive statistics and correlation between analyzed variables can be found in Table 2. Hierarchical regression, which was conducted, explains 29% of variances for life satisfaction ($R^2 = 0.29$, $F = 10.87$, $p < 0.01$) and 36% of variances for depressive symptoms ($R^2 = 0.36$, $F = 15.33$, $p < 0.01$). Detailed findings are presented in Tables 3 and 4. Our analysis shows that in the final step of hierarchical regression analysis, symptoms of inattention ($\beta = -0.14$, $p < 0.01$), and social camouflaging ($\beta = -0.14$, $p < 0.01$) were significantly negatively related to life satisfaction. Furthermore, being in a relationship ($\beta = 0.10$, $p < 0.05$) and having a subjectively good financial situation ($\beta = 0.37$, $p < 0.01$) positively predicted life satisfaction. Regarding depressive symptoms, our research has shown that social camouflaging ($\beta = 0.18$, $p < 0.01$), having diagnosed depression ($\beta = 0.14$, $p < 0.01$) or anxiety disorder ($\beta = 0.11$, $p < 0.01$) and hyperactivity-impulsivity ($\beta = 0.32$, $p < 0.01$) or inattention symptoms ($\beta = 0.23$, $p < 0.01$) were significant positive predictors of depressive symptoms. Being in a relationship ($\beta = -0.13$, $p < 0.01$) was negatively related to depressive symptoms.

Table 2 Descriptive statistics and correlation between analyzed variables

Variables	M	SD	[1]	[2]	[3]	[4]
[1] Hyperactivity-impulsivity symptoms	25.78	4.75	--			
[2] Inattention symptoms	27.94	3.93	0.365*	--		
[3] Depressive symptoms	14.52	5.82	0.425*	0.384**	--	
[4] Life satisfaction	17.56	6.62	−0.134**	−0.238**	−0.513**	--
[5] Camouflaging	4.26	1.34	0.151**	0.141*	0.270**	−0.212**

* - $p < 0.05$, ** - $p < 0.01$

Table 3 Results of hierarchical regression analysis for age, age of diagnosis, other diagnoses (ASD, depression, anxiety disorder, personality disorder), lack of psycho- and pharmacotherapy, subjective evaluation of financial situation, being in a relationship, hyperactivity-impulsivity, inattention, camouflaging as predictors of life satisfaction

Life satisfaction	Step 1					Step 2					Step 3				
	BE	SE	β	t	p	BE	SE	β	t	p	BE	SE	β	t	p
Age	−0.130	0.124	−0.159	-1.051	0.294	−0.122	0.122	−0.148	−0.997	0.320	−0.147	0.121	−0.179	-1.213	0.226
Age of diagnosis	0.044	0.124	0.054	0.357	0.721	0.047	0.122	0.057	0.384	0.701	0.066	0.120	0.080	0.547	0.585
Diagnosed ASD	−0.161	0.925	−0.009	−0.174	0.862	−0.328	0.907	−0.018	−0.361	0.718	0.141	0.911	0.008	0.155	0.877
Diagnosed depression	-1.328	0.760	−0.099	-1.746	0.082	-1.520	0.745	**−0.114***	-2.039	**0.042**	-1.442	0.737	−0.108	-1.956	0.051
Diagnosed anxiety disorder	−0.620	0.774	−0.045	−0.800	0.424	−0.369	0.761	−0.027	−0.485	0.628	−0.306	0.753	−0.022	−0.406	0.685
Diagnosed personality disorder	−0.059	1.523	−0.002	−0.038	0.969	0.114	1.490	0.004	0.076	0.939	−0.006	1.474	0.000	−0.004	0.997
Lack of psycho- and pharmacotherapy	-1.714	1.092	−0.078	-1.569	0.118	-1.832	1.069	−0.083	-1.715	0.087	-1.915	1.057	−0.087	-1.811	0.071
Subjective evaluation of financial situation	2.073	0.257	**0.403****	8.053	**< 0.001**	1.961	0.255	**0.383****	7.703	**< 0.001**	1.891	0.253	**0.368****	7.479	**< 0.001**
Being in a relationship	1.306	0.744	0.089	1.756	0.080	1.451	0.729	**0.0.98***	1.990	**0.047**	1.426	0.720	**0.097***	1.979	**0.049**
Hyperactivity-impulsivity symptoms						−0.122	0.072	−0.087	-1.683	0.093	−0.095	0.072	−0.068	-1.316	0.189
Inattention symptoms						−0.249	0.089	**−0.148****	-2.816	**0.005**	−0.228	0.088	**−0.136****	-2.599	**0.010**
Camouflaging											−0.709	0.246	**−0.143****	-2.888	**0.004**
R^2	0.235**					0.277**					0.295**				
ΔR^2	0.236**					0.038**					0.019**				
F	10.933**					10.852**					10.874**				

*- $p < 0.05$, **- $p < 0.01$

Discussion

Our research shows that social camouflaging is negatively related to life satisfaction and positively to depression symptoms in women with ADHD even after adjusting for other demographic variables. This aligns with findings in the literature on autistic individuals (Field et al., 2024; Hull et al., 2021; Bradley et al., 2021), suggesting that social camouflaging serves similar psychological functions across these groups. From the perspective of Gender Schema Theory (Bem, 1981; Salciunas, 2021), societal norms about 'appropriate' female behavior may pressure women to camouflage ADHD symptoms. These gendered expectations, deeply internalized from a young age, often emphasize traits such as compliance, good behavior, calmness, and submissiveness in girls, which conflicts with ADHD symptoms, leading women to feel a need to mask these traits to fit societal expectations. In addition, consistent with Goffman's seminal work on stigma and the 'spoiled identity' (1963; Bos et al., 2013), women with ADHD may use social camouflaging as a strategy to mitigate the psychosocial challenges and stigma and avoid social exclusion. Awareness of public stigma often leads to self-stigma, where negative stereotypes are internalized and have a negative impact on self-esteem and well-being (Bos et al., 2013). In effect, the act of camouflaging to 'pass' as normal within societal norms underscores the perpetual struggle between concealing one's stigmatized identity and the ongoing threat of being discredited. This tension may introduce significant psychological stress, particularly in scenarios where disclosure might lead to increased social rejection (Bos et al., 2013). Social camouflaging may not only be threatening, exhausting and cognitively demanding but can also make one feel as if they are hiding their true self (Field, 2024; Mylett, 2022). In effect, it may lead to feelings of inauthenticity and identity conflict. According to self-determination theory (SDT), acting authentically is crucial for well-being (Ryan et al., 2019). Social camouflaging may undermine this authenticity by impacting all three basic psychological needs posited by SDT: autonomy, competence, and relatedness. By masking their true selves, individuals may compromise their autonomy, perceive themselves as less competent, and struggle to form genuine connections, all of which are essential for psychological health. Consequently, the failure to meet these basic needs can lead to increased anxiety, stress, and depression.

Consistent with previous research (Ogg et al., 2016), our study found that inattention symptoms were associated with both lower life satisfaction and higher depressive symptoms. Furthermore, symptoms of hyperactivity-impulsivity were positively related to depressive symptoms, but not to life satisfaction. This differentiation highlights the nuanced impact of different ADHD symptom dimensions on mental well-being. Moreover, in line with previous research (Medgyesi & Zólyomi, 2016) the subjective evaluation of one's financial situation was positively related to life satisfaction.

Our results also show that being in a relationship can have a positive effect both on life satisfaction and the prevention of depressive symptoms in women with diagnosed ADHD. These findings support previous research showing that being in a relationship can mitigate depressive symptoms (Whitton et al., 2013) and increase life satisfaction partly due to the sense of social support and love it provides (Facio & Resett, 2014; Yalçin, 2011).

Limitations

Our study's findings are limited by its reliance on self-report measures, the cross-sectional design, and participants' self-reported ADHD diagnosis, which may affect the generalizability and interpretability of the results. The majority of the survey's recipients were women with a higher education degree and from big cities. Consequently, the results can not be generalized for the whole Polish female population. It is needed to conduct further studies with a more diverse sample, also including women from smaller towns and with lower education levels. Additionally, the predominance of highly educated women from big cities in the study may reflect greater access to diagnosis and a better understanding of ADHD and its symptoms in these areas. Our new scale for measuring social camouflaging in ADHD, still in its early validation stages, might not yet fully grasp the complex nature of social camouflaging in individuals with ADHD e.g. it does not distinguish various dimensions of social camouflaging (compensation, masking, and assimilation) that were found in autism research. Moreover, it primarily focuses on the masking of hyperactivity-impulsivity symptoms, with less emphasis on the camouflaging of inattention symptoms. These shortcomings may impact the reliability and validity of our findings and suggest a need for further refinement of the measure to better represent the diverse experiences of social camouflaging in ADHD. To address these gaps, future research should expand the validation process of the scale across varied populations and settings to ensure its robustness and applicability. Additionally, integrating qualitative research methods could significantly enhance our understanding of social camouflaging. Detailed, qualitative accounts of how women with ADHD manage their symptoms in different social situations would enrich the

Table 4 Results of hierarchical regression analysis for age, age of diagnosis, other diagnoses (ASD, depression, anxiety disorder, personality disorder), lack of psycho- and pharmacotherapy, subjective evaluation of financial situation, being in a relationship, hyperactivity-impulsivity, inattention, camouflaging as predictors of depressive symptoms

PHQ-9	Step 1					Step 2					Step 3				
	BE	SE	β	t	p	BE	SE	β	t	p	BE	SE	β	t	p
Age	0.025	0.119	0.034	0.209	0.835	0.037	0.103	0.051	0.359	0.720	0.064	0.101	0.089	0.637	0.525
Age of diagnosis	−0.017	0.118	−0.023	−0.142	0.887	−0.053	0.102	−0.073	−0.520	0.604	−0.074	0.100	−0.102	−0.738	0.461
Diagnosed ASD	-1.118	0.883	−0.068	-1.266	0.206	−0.608	0.762	−0.037	−0.798	0.426	-1.119	0.759	−0.069	-1.474	0.141
Diagnosed depression	1.335	0.726	0.113	1.838	0.067	1.768	0.626	**0.150****	2.822	**0.005**	1.683	0.614	**0.143****	2.740	**0.006**
Diagnosed anxiety disorder	1.860	0.739	**0.153***	2.516	**0.012**	1.437	0.639	**0.118***	2.248	**0.025**	1.368	0.627	**0.112***	2.183	**0.030**
Diagnosed personality disorder	0.994	1.454	0.037	0.683	0.495	0.710	1.252	0.026	0.567	0.571	0.841	1.227	0.031	0.685	0.494
Lack of psycho- and pharmacotherapy	0.534	1.043	0.028	0.512	0.609	0.820	0.898	0.042	0.913	0.362	0.910	0.880	0.047	1.034	0.302
Subjective evaluation of financial situation	−0.418	0.246	−0.092	-1.699	0.090	−0.278	−0.248	−0.055	-1.158	0.248	−0.171	0.211	−0.038	−0.813	0.417
Being in a relationship	-1.508	0.710	**−0.116***	-2.123	**0.035**	-1.750	0.612	**−0.135****	-2.858	**0.005**	-1.724	0.600	**−0.133****	-2.873	**0.004**
Hyperactivity-impulsivity symptoms						0.428	0.061	**0.349****	7.023	**< 0.001**	0.398	0.060	**0.325****	6.622	**< 0.001**
Inattention symptoms						0.362	0.074	**0.244****	4.865	**< 0.001**	0.329	0.073	**0.229****	4.635	**< 0.001**
Camouflaging											0.773	0.204	**0.177****	3.779	**< 0.001**
R^2	0.098**					0.336**					0.365**				
ΔR^2	0.098**					0.238**					0.029**				
F	3.863**					14.599**					15.133**				

*- $p < 0.05$, **- $p < 0.01$

quantitative data, providing a more nuanced view of the impacts of social camouflaging.

Clinical Implications

Social camouflaging may significantly impact mental health and complicate the clinical presentation of ADHD symptoms, leading to delayed diagnoses. Our data indicate that most women were diagnosed with ADHD in adulthood, likely due to long-term camouflaging behaviors and cultural factors prevalent in Poland, where neurodevelopmental disorders are often misunderstood and underdiagnosed. These findings emphasize the need for clinicians to assess both ADHD symptoms and camouflaging behaviors. The refinement of the Social Camouflaging ADHD Scale is crucial to accurately measure these behaviors, enhancing diagnostic precision and enabling tailored treatment strategies. Clinicians require specific training to detect social camouflaging effectively. Additionally, educational support systems must adapt to better accommodate neurodiverse students and minimize the need for camouflaging ADHD symptoms. Public health policies should focus on reducing ADHD stigma and increasing awareness of neurodiversity, particularly among women. Effective social support systems, including online and in-person support groups, can provide essential networks for women with ADHD, promoting social inclusion and mitigating the psychological burden of camouflaging. Finally, the development and evaluation of therapeutic and preventive interventions specifically designed for women with ADHD are essential. These interventions should address social camouflaging and support individuals at risk.

Conclusions

Our research contributes to an enhanced understanding of Attention Deficit Hyperactivity Disorder (ADHD) in women, examining the interaction between social camouflaging, demographic variables, and mental well-being. The findings indicate that women with ADHD may experience a psychological cost due to their efforts to mask symptoms and conform to societal norms. This study highlights the importance of recognizing social camouflaging as an important factor that may affect the mental health of women with ADHD We advocate for further research across diverse age groups, genders, and cultural settings, emphasizing the need for longitudinal studies and mixed-methods research to deepen our understanding of camouflaging behaviors. Our findings also underscore the need for targeted interventions that reduce societal pressures and promote neurodiversity acceptance. Public health campaigns should educate about ADHD in women to reduce stigma and support early diagnosis. Healthcare training should include strategies to address the unique challenges of social camouflaging in women with ADHD. Moreover, addressing broader risk factors such as low socioeconomic status and the absence of supportive relationships can further improve well-being of women with ADHD.

Supplementary Information The online version contains supplementary material available at https://doi.org/10.1007/s10803-024-06410-6.

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Declarations

Ethical Approval This study was approved by the institutional review board of the Faculty of Psychology, University of Warsaw. All procedures performed in the study were done so in accordance with all applicable ethical standards, including the 1964 Helsinki declaration and its later amendments or comparable ethical standards.

Informed Consent Informed consent was obtained from all participants included in the study.

Conflict of Interest We wish to confirm that there are no conflicts of interest associated with this publication and there has been no significant financial support for this work that could have influenced its outcome.

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